



PROCEDURE

Clinical Observation

Scope (Staff):	NMHS Mental Health (MH) Clinical Staff
Scope (Area):	NMHS MH Inpatient Areas

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1. Aim

North Metropolitan Health Service (NMHS) Mental Health (MH) is committed to providing safe clinical care that optimises recovery of the patient in the least restrictive manner. Nurses undertake routine clinical observations of patients in the course of their work to monitor health parameters and response to treatments. In in mental health inpatient settings **Clinical Observation Levels** are undertaken by nurses for all patients as determined by the Consultant (or delegate) in consultation with nursing staff. Nurses monitor and report on clinical observations in accordance with the requirements for each level to manage identified risks and ensure patient safety and wellbeing. Reviews are undertaken by medical staff in accordance with the frequency required for each patient at each level in the service.

2. Background

The degree of concern about a patient's safety is a key factor in determining where the patient is managed and how their care is coordinated in accordance with the [Department of Health \(DoH\) Safety Planning for Mental Health Consumers Policy](#) and the [Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Services in Western Australia](#).

Decisions regarding clinical observation requirements will be influenced by the severity of the patient's illness, their level of insight, treatment compliance, vulnerability, degree of impulsivity, the presence of active suicidal ideation and their state of emotional arousal. Refer to the [MH Clinical Risk Assessment and Management Policy](#), the [MH Sexual Safety in Inpatient Services Guideline](#), the [MH Person of Concern Protocol](#) and the [MH Recognising and Responding to Acute Deterioration \(RRAD\) Procedure](#).

3. Risk

Risks may include risk of harm to self or others and or medical risk leading to poor clinical outcomes and potential risk to the organisation.

Non-compliance with this procedure will impact on the following risk categories:

Culture	☒	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Principles

The clinical handover for each patient must include the minimum requirements for safe and effective clinical handover and communicate ongoing care requirements that includes mental health observations and any risk and behavioural assessments undertaken with risks clearly identified. Refer to the [DoH Clinical Handover Policy](#).

The use of increased nursing clinical observations is one of many interventions undertaken to monitor change, escalate care in a timely manner in response to clinical deterioration and maintain patient safety.

Undertaking clinical observation of patients has the potential to be intrusive, particularly when required for prolonged periods and should be undertaken sympathetically and only when assessed as necessary for the welfare of the patient. This process requires nurses to develop a therapeutic alliance with patients while being mindful of fluctuations in mental state and the potential for mental and physical deterioration requiring ongoing assessment of risk.

The use of a “special” refers to the allocation of a designated nurse to provide additional observation, assessment and monitoring of health-related parameters as directed by medical staff and as documented in the integrated notes, the Treatment Support and Discharge Plan (TSDP) and the MH Clinical Observation Form PMR29 (**Appendix 3**).

Physiological, behavioural and mental state parameters are assessed, monitored and reviewed in accordance with the following requirements;

- [MH Clinical Risk Assessment and Management Policy](#)
- [MH Recognising and Responding to Acute Deterioration Procedure](#)
- [MH Physical Healthcare Management Policy](#)
- [DoH Triage to Discharge Policy Framework](#)

Clinical observations are recorded on the following documents:

- [DoH Statewide Standardised Clinical Documentation](#) (SSCD)
- MH Clinical Observation Form (PMR29)
- MH Agitation and Arousal PRN Medication Chart (PMR60)
- MH Observation and Response Chart (ORC) (PMR38)

5. Levels of Clinical Observation

Clinical observation requirements are specified in the medical record by the treating doctor and management plans are updated to include observations levels, the number of staff required and any additional requirements. This information is recorded by nursing staff on the PMR29 that confirms the Clinical Observation level as ordered, date and time commenced and ceased, clinical observations and names and designations of staff responsible for undertaking clinical observations.

Level 1	Clinical Observation Level 1 is identified as a 1:1 nurse special (referring to the ratio of nurses in attendance per patient) and may be undertaken at “arm’s length” or as a line of sight “visual” observation as prescribed in the medical record. In certain circumstances a second nurse (or staff member) may be required to manage the clinical risk and this is documented as a 2:1 nurse special.
Level 2	Clinical Observation Level 2 requires a nurse to undertake observations of identified patients usually at 10, 15 or <u>30-minute intervals</u> as prescribed in the medical record until reviewed by the medical team.

Level 3	Clinical Observation Level 3 requires a nurse to undertake clinical observations of identified patients at <u>one-hour intervals</u> as prescribed in the medical record until reviewed by the medical team.
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6. Undertaking Clinical Observations

Clinical observations are undertaken by a nurse and is assessed by the nurse manager or delegate as having the skills required to undertake the responsibilities of the role as determined by the nature of the risk/s identified. The admitting Consultant, delegate or person initiating increased observations will determine the level of clinical observations that are required and complete the PMR29 to if it is necessary for a nurse to undertake Clinical Observation Level 1 either visual or at arm's length.

The decision to allocate a nurse will vary depending on the degree of complexity and clinical care needs and risk factors identified:

- risk to self;
- risk to others;
- risk of vulnerability;
- risk associated with behavioural disturbance /changes in mental state/cognitive changes; and
- medical risk (e.g., altered physiological state, invasive treatments, falls).

When an extra staff member is required in addition to the nurse undertaking clinical observations, they will be allocated depending on the skillset required and the continuing care requirements. Following review consider the first special to be allocated from existing staffing numbers with relevant approval sought for additional resources required.

The treating Consultant (or delegate) or after-hours Duty Psychiatrist will conduct a face-to-face assessment of the patient to specify the risk(s) identified and determine the clinical observation level required and any other requirements (such as physical health parameters, access to family and environmental factors).

The shift co-ordinator will allocate a nurse to undertake clinical observations, receive a clinical handover and review the patient medical record prior to commencing clinical observations.

The allocated nurse will review the patient medical record, check that the required documentation is available and accurately and contemporaneously records clinical observations for the period allocated (Refer to **Appendix 1: Clinical Observation Levels Flowchart**)

Whenever possible, rotations for nurses undertaking clinical observations should be implemented, with no more than two hours undertaken consecutively unless there are clinical reasons to alter the intervals. Nurses undertaking clinical observations should endeavour to support and engage with patients to build therapeutic alliances while being responsive to patient needs, monitoring changes in their physiological and mental state, completing risk assessments and escalating care when required. Care provided is documented by the nurse when care is handed over to another nurse during shifts and at the end of each shift (Refer to **Appendix 3: PMR29**).

6.1 Clinical Observation Level 1

Clinical Observation Level 1 requires that nurses are always within arm's length of the patient or in constant line of sight of the patient depending on the medical order.

- *Within arm's length* – this level of clinical observation is usually ordered when a patient is identified as being at risk or vulnerable due to unpredictable, impulsive, self-harming behaviours, suicidal ideation or other disorganised behaviours.
- *Visual* – this level of clinical observation is used for identified risks that do not require a nurse to remain in close proximity at all times or in situations where having a nurse in constant close proximity is contributing to the patient's agitation or ability to rest and risk can be safely managed by the nurse always remaining in direct line of sight of the patient.
- Clinical Observation Level 1 is often prescribed for patients who are actively suicidal. The patient may be expressing suicidal intent or may recently have carried out an act of deliberate self-harm or engaged in risky, impulsive behaviours.
- Once a decision is made to place a patient on Clinical Observation Level 1 it is necessary for the treating or Duty Psychiatrist (or medical delegate) to document the requirement for a 1:1 special and complete the PMR29.
- On occasions and when acuity and/or risks are elevated and unable to be managed in a less restrictive manner it may be necessary for decision to be made to order and allocate two staff to undertake clinical observations (2:1).

It is the responsibility of the nurse in charge of the shift, to allocate the nursing care of the patient on Clinical Observation Level 1 to the most appropriate nurse and to ensure that increased observations levels are communicated in clinical handovers in accordance with the requirements of the [WA DoH Clinical Handover Policy](#), the ISOBAR framework and the [Role of the Allocated Nurse in MH Inpatient Services Procedure](#).

- Following handover, it is the responsibility of the individual nurse taking over the care of the patient to confirm the type of Clinical Observation Level 1 prescribed in the medical record by reviewing the clinical documentation, observation charts and PMR29.
- Patients on Clinical Observation Level 1 will have the observation checks including the respiratory rate recorded by the allocated nurse as specified on the Clinical Observations Form (**PMR29**) with frequency of observations noted. Times of observations must be recorded accurately and contemporaneously.
- When patients on Clinical Observation Level 1 are toileting or showering, their needs for safety may restrict their privacy needs. Maintaining their safety must be the priority and all decisions relating to increasing or decreasing clinical observations must be discussed with the Shift Coordinator and/or Consultant or delegate and must be clearly documented in the Medical Record.
- Patients on Clinical Observations Level I are required to be reviewed by the Consultant or delegate every 24 hours in acute areas.
- Patients on Clinical Observations Level I in Hospital Extended Care wards are required to be reviewed by the Consultant (or delegate) at a frequency decided on a case-by-case basis with the clinical team.

6.2 Clinical Observation Level 2

- Patients on Clinical Observation Level 2 must be checked at a minimum of every 30 minutes or as prescribed. The frequency of observations will be documented in the Integrated Progress Notes, with explanation, by the treating or Duty Psychiatrist (or medical delegate) following



face to face assessment of the patient and in consultation with the nurse in charge of the shift. If the need for more frequent checks is identified placing the patient on Clinical Observations Level 1 will be considered.

- Patients must have observation checks, including the respiratory rate, recorded on the MH Clinical Observation Form (PMR29) and frequency of observations noted. Times of observations must be recorded accurately and contemporaneously.
- Observation checks **must not** be recorded in advance.
- Patients may be restricted to specific areas of the ward depending on the nature of the risk and the interests or preferences of the patient. This should be discussed with the patient and be included in the TSDP.

6.3 Clinical Observation Level 3

Clinical Observation Level 3 requires a nurse to undertake clinical observations of identified patients at one-hour intervals as prescribed in the medical record or until reviewed.

Patients not prescribed Clinical Observation Level 1 or 2 will be prescribed Clinical Observation Level 3 as documented by the treating or Duty Psychiatrist (or medical delegate) in their medical record and PMR29.

- The nurse caring for the patient on Clinical Observation Level 3 will:
 - Check the observation level in the medical record and discuss any documented restrictions with the patient.
 - Check and document that the patient is present in the ward area at a minimum of one hourly interval.
 - Check that the patient description is documented in the Medical Record and the description is current.
 - Record the observations checks on the PMR29 accurately and contemporaneously.
 - Check that the patient is present at mealtimes and when the ward is secured for the night and document as required (Refer to **Appendix 3: PMR29**).
 - Document any incidents of non-attendance and absence in the Progress Notes (PMR7) and escalate in accordance with the agreed service protocol.
 - Notify the nurse in charge of the shift immediately if the patient cannot be accounted for at agreed/specified time. Refer to the [MH Missing or Suspected Missing Patients Procedure](#).
 - Ensure that an update is provided on the whereabouts and well-being of every patient in their care as part of the nursing communication handover process at the end of each shift.

6.4 Clinical observations at night-time

Clinical observations are recorded at specified intervals during the night shift. Respiration rates must be recorded at a minimum of 1-hour intervals throughout the night when patients are asleep.

6.5 Clinical observations during periods of seclusion

Refer to clinical observation requirements in accordance with [MH Seclusion Policy](#) and the [WA Chief Psychiatrist's Guidelines](#) advising that the use of CCTV is not a substitute for conducting face-to-face clinical observations



6.6 Clinical Observations for patients under 18

All patients under 18 years admitted to a NMHS MH Inpatient Service (including Mental Health Observation Areas) must have documented strategies to keep the child safe in an Adult Mental Health facility as identified by the Admitting Consultant Psychiatrist in consultation with the child, parent and/or guardian and nursing staff.

Segregation of Children from Adult Inpatients Notification Form must be completed during admission and should contain the following information:

- The reasons why the person in charge is satisfied that the mental health service can provide the child with treatment, care and support that is appropriate having regard to the child's age, maturity, gender, culture and spiritual beliefs.
- The measures that the mental health service will take to ensure that, while the child is admitted as an inpatient, the child is protected and the child's individual needs in relation to treatment and care are met ([Refer to the Mental Health Act 2014 \(MHA 2014\) s. 303 Segregation of children from adult inpatients, Office of Chief Psychiatrist, WA](#)).
- The management plan must be documented by the Consultant in the medical record and staff, parents and guardians should be satisfied that the strategies that are in place will be effective in keeping the child safe.

The *MHA 2014* s.303 is flexible around what actions may be taken by the inpatient mental health service to comply and does not specify 1:1 special however the Chief Psychiatrist of WA reports that the use of a 1:1 special is a strategy commonly used for ensuring the safety of a child under 18 years in an adult mental health facility.

6.7 Consultation and decision making

- The nurse in charge of the ward may place a patient on Clinical Observation Level 1 or Level 2 as an emergency measure while the treating or Duty Psychiatrist (or medical delegate) is requested to undertake an urgent face to face clinical review.
- The final decision to place a patient on Clinical Observation Level 1 or Level 2 is the responsibility of the treating or Duty Psychiatrist (or medical delegate) and the least restrictive clinical observation level consistent with patient safety should be prescribed.
- Decisions regarding clinical observation levels are made by the treating or Duty Psychiatrist (or medical delegate) in consultation with the patient, carer or nominated Personal Support Person where practicable following discussion with nursing staff.
- When the Clinical Observation Level 1 or Level 2 is prescribed the responsible treating or Duty Psychiatrist (or medical delegate) will personally examine/review the patient (face-to-face) and, in consultation with the Nurse Manager (or delegate) determine the level of observation required.
- Changes in clinical observation levels must be documented by the treating or Duty Psychiatrist (or medical delegate) in the medical record at the time of their decision and the TSDP updated.
- The treating or Duty Psychiatrist (or medical delegate) will discuss the reasons for ordering Clinical Observation levels and safety concerns with the patient.
- Patients may request to utilise strategies to maintain their safety during admission as part of coping and safety awareness planning, for example having access to low stimulus areas or a bedroom close to the staff workstation or having a nurse accompany them when they feel unsafe or at an increased risk of self-harm while moving around the ward to access facilities (Refer to **Appendix 4: Agitation and Arousal Form PMR60**).



6.8 Clinical handover and transfer of care

The clinical handover process should identify patients who are at risk and provide specific information relevant to their ongoing management, including the need for an ongoing nurse special (Refer to [MH Clinical Risk Assessment and Management Policy](#)).

- Nursing staff will ensure that the clinical handover for each patient includes identification of risk, clinical observation level and any restrictions or special care needs in accordance with the requirements of the [DoH Clinical Handover Policy](#).
- Clinical Handover provides opportunity to communicate relevant patient information to occur at the point of handover and includes the transfer of information at end of shift, when intra or inter hospital transfer occur and at point of discharge.

6.8.1 Transfer of Care (inter/intra hospital)

- Consideration should be given to whether the patient is accompanied by a nurse from the referring hospital who is familiar with the patient and their management to ensure continuity of care.
- The clinical observation level and nursing observation requirements are identified including the need for an ongoing nurse special.
- The nurse accompanying the patient delivers the clinical documentation and provides a comprehensive clinical handover to the receiving service and/or clinical team.
- Documents provided when a patient is transferred to another hospital includes the following:
 - MHA 2014 [Form 4A](#) Transport Order;
 - MHA 2014 [Form 4C](#) Transfer to another Hospital;
 - Patient Transfer Summary ;
 - Progressive Risk Assessment Sticker;
 - Current Risk Assessment Management Plan (RAMP) with risks identified;
 - Treatment Support and Discharge Plan (TSDP);

7. Review of Clinical Observation Levels

All patients on clinical observation levels will have a TSDP completed which will be reviewed as part of the multidisciplinary team review process.

7.1 Review of patients on Observation Level 1

- Patients on Clinical Observation Level 1 will be reviewed (face-to-face) with a risk assessment documented by the treating or Duty Psychiatrist or delegate every 24 hours. Where possible this review should occur daily prior to 11am. (**Refer to Appendix 2: Medical Review of Clinical Observation Level 1 Flowchart**).
- Patients may be approved for continuation of Clinical Observation Level 1 to cover a weekend period up to 72 hrs. This requires the approval of the Treating Psychiatrist or delegate and must be documented in the patient's medical record.
- Continuation of a 1:1 special for specified periods greater than 72 hrs must be obtained and documented in the patient's medical record with a management plan and expected duration of treatment (**Refer to Appendix 2**).
- Patients who require the allocation of a nurse special to undertake Clinical Observations Level 1 as part of a long-term management plan will require a comprehensive multidisciplinary management plan with allocated team responsibilities and specified review times.

-
- Approval to continue Clinical Observation Level 1 for a period greater than 28 days must be discussed with the Medical Co-Director, the Director of Medical Services and the Director of Nursing to explore all available resources and options.
 - Ongoing risk assessments detailing the identified risk(s) and patient's response to the management plan must be completed and documented in the patients' medical record.

7.2 Responsibilities of the nurse undertaking clinical observation levels.

- The name of the nurse undertaking Clinical Observation Level 1 will be identified on the allocations board in the nurse station and communicated in accordance with safety protocols for the service.
- Nurses allocated to a patient on Clinical Observation Level 1 will ensure that other staff know of their whereabouts at all times and will not leave the ward area with the patient except for pre-arranged appointments in consultation with the Nurse Manager (NM), Clinical Nurse Specialist (CNS) or delegate.
- Nurses will remain with a patient on Clinical Observation Level 1 during interviews or any clinical interventions involving other clinicians or family /carers.
- The nurse undertaking clinical observation levels (1-3) will report on clinical deterioration (mental, physical, behavioural) and record observations accurately and contemporaneously detailing the patient's mental state and risk factors.

7.3 Requirements for Extra Nursing Staff

When a patient has been prescribed Clinical Observation Level 1 the CNS, Clinical Nurse Manager (CNM), NM or, if after hours, the nurse in charge of the shift will assess the wards staffing needs. The determination as to whether extra nursing staff are required should include consideration of the following (Refer to Risk Assessment Tool to Report and Request Extra Staff):

- Number of nurses on the shift and skill mix (including Assistants in Nursing (AIN))
- Number of patients and their overall acuity and dependency (refer to acuity documentation if completed).
- Number of patients on Clinical Observation Level (1-3).
- Number of patient movements (admissions, discharges, transfers, escorts and appointments).
- Scheduled programs and activities requiring nurses to participate.
- Environmental factors.

7.3.1 Approval process during normal hours

The NM (or delegate) will assess the need for an extra nurse following consultation with nursing staff, an assessment of patient needs, overall acuity levels and nursing resources and escalates to the Coordinator of Nursing for approval. Consideration will be given to covering the first special out of the ward nursing complement.

7.3.2 Approval process after hours including weekends

The Clinical Nurse Manager (CNM) (or delegate) will assess the need for an extra nurse following consultation with nursing staff and an assessment of patient needs, overall acuity levels and available nursing resources and escalates to the on-call Coordinator of Nursing for approval. Consideration will be given to covering the first special out of the ward nursing complement.

8. Documentation

On admission each patient will have a documented Risk Assessment and Management Plan (RAMP) (**SMHMR905**) which records the clinical observation level prescribed and includes the rationale for the observation level by identifying the nature of the risk(s).

The Treatment Support and Discharge (**SMHMR907**) and the Consumer Safety Plan (**SMHMR932**) will be completed and strategies identified with the patient will be recorded.

Refer to the [MH Clinical Risk Assessment and Management Policy](#), the [MH Management of Clinical Alerts Policy](#) and the [MH Clinical Record Documentation Procedure](#) for further information.

Clinical Documentation identifies the level of clinical observation as authorised, number of staff required, the frequency of observations and for Clinical Observation Level 1 whether visual or within arm's length is required and if any change is to occur overnight.

Any other factors which may increase the risk(s) and any restrictions or circumstances of concern including environmental risk(s) are identified.

The **PMR29 (Appendix 3)** is completed for identified patients on each shift as required.

Clinical information relating to each patient is recorded in the integrated Progress Notes (**PMR7**) at a minimum of once per shift in accordance with the [MH Clinical Record Documentation Procedure](#). This information includes a review of mental state, risk assessment, level of clinical observation required and other prescribed interventions and updates to the management plan as a result of a medical review or the multidisciplinary team review (Refer to the [MH Clinical Risk Assessment and Management Policy](#)).

Documentation of clinical observation is recorded on the **PMR29** and is filed in the patient's medical records when it is completed.

9. Compliance, monitoring and evaluation

- Clinical documentation audits are undertaken as part of the Safety Quality and Performance (SQP) annual audit calendar, outcomes forwarded to the Leadership Teams for review and actions as required.
- Audit reports and actions plans are tabled at the MHET and Executive Clinical Governance Committee.

Related internal policies, procedures and guidelines

[MH Medication: Prescribing, Administration and Management for Inpatients Policy](#)

[MH Missing or Suspected Missing Patients Procedure](#)

[MH Clinical Risk Assessment and Management \(CRAM\) Policy](#)

[MH Clinical Record Documentation Procedure](#)

[MH Management of Clinical Alerts Policy](#)

[MH Person of Concern Protocol](#)

[MH Recognising and Responding to Acute Deterioration \(RRAD\) Procedure](#)

[MH Physical Healthcare Policy](#)

[MH Sexual Safety in Inpatient Services Policy](#)

[MHPHDS Duress Alarm Policy](#)

[NMHS Clinical Documentation Policy](#)

[NMHS Recognising and Responding to Acute Deterioration \(RRAD\) Policy](#)

[NMHS Falls Prevention and Management Policy](#)

References

[DoH Clinical Handover Policy](#)

[DoH Safety Planning for Mental Health Consumers Policy MH 0181/24](#)

[DoH Agitation and Arousal PRN Medication Chart](#)

[DoH WA Clinical Handover Guideline](#)

[DoH Recognising and Responding to Acute Deterioration Policy](#)

[DoH Triage to Discharge Mental Health Framework for Statewide Standardised Clinical Documentation \(SSCD\)](#)

[Chief Psychiatrist's Standards for Clinical Care](#)

[Chief Psychiatrist's Guidelines](#)

[Chief Psychiatrist's Sexual Safety Guidelines](#)

[Mental Health Act 2014](#)

[ACSQHC Advisory AS19/01 Recognising and Responding to Acute Deterioration Standard: Recognising Deterioration in a Person's Mental State](#)

[ACSQHC Acute Anaphylaxis Clinical Care Standard](#)

[ACSQHC Fact Sheet Preventing Pressure Injuries and Wound Management](#)

[ACSQHC Evidence for the Safety and Quality Issues Associated with the Care of Patients with Cognitive Impairment in Acute Care Settings: A Rapid Review](#)

[ACSQHC Delirium Clinical Care Standard](#)

[ACSQHC Sepsis Clinical Care Standard](#)

[ACSQHC Cognitive Impairment](#)

Forms and other related documents

MH Integrated Progress Notes PMR7

MH Clinical Observation Form PMR29

MH Agitation and Arousal Chart PMR60

OCP Segregation of Children from Adult Inpatients Notification Form (MHA section 303)

MHA 2014 Form 4A Transport Order

MHA 2014 Form 4C Transfer to another Hospital

Patient Transfer Summary

Progressive Risk Assessment Sticker

Policy Sponsor	Director of Nursing NMHS MHPHDS				
Policy Contact	Director of Nursing NMHS MHPHDS				
Date First Issued:	13/04/2011	Last Reviewed:	17/10/2022	Review Date:	17/10/2025
Approved by:	Director of Nursing NMHS MHPHDS			Date:	29/05/2024
Endorsed by:	Executive Director NMHS MHPHDS			Date:	31/05/2024
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety ☐  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☒  Std 8: Recognising and Responding to Acute Deterioration				
Chief Psychiatrist's Standards for Clinical Care:	Assessment Care Planning Physical Health Care of Mental Health Consumers Risk Assessment and Management Seclusion and Bodily Restraint Reduction				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Document Version Control			
Date	Version	Author	Notes
31/08/2017	2.0	Adrian Redknap, Acting Clinical Nurse Coordinator.	Inclusion of need for medical staff to identify and clearly document the type and level of risk in progress notes. Changes to terminology. Updated template.
25/11/2017	2.1	Deputy Executive Director/Executive Director of Nursing	Clarification of roles to conduct observations, observations of patients under the age of 18 years, clarification of internal and external reviews.
23/01/2018	2.2	Deputy Executive Director/Executive Director of Nursing	Inclusion of respiration rates to be taken and frequency of recording Clinical Observation levels in Policy; clarification of frequency of collection and recording to be contemporaneous in Procedures: Clinical Observation Level 3.
29/08/2018	2.3	Deputy Executive Director/Executive Director of Nursing	Amendments (sponsor approved): Use of Clinical Visual Observations Checklist or PMR29 to document clinical visual observations included. Inclusion of Clinical Visual Observations Checklist as appendix. Clarification of Clinical Observation Level 2, Inclusion of Aboriginal Impact Statement. Updated template & references
09/10/2018	2.4	A/Policy & Audit Officer	Inclusion of NMHS MH and higher-order Clinical Care of People who may be Suicidal policy documents.

06/11/2018	2.5	Clinical Nurse Consultant, NMHS MH Older Adult	Minor amendments to reflect the creation of the NMHS MH CRAM Policy.
17/10/2022	3.0	Nurse Director	New template, converted from policy to procedure, amendments to reflect current best practice and principles
22/11/2022	3.1	Director of Nursing MHPHDS	Minor amendment, hyperlinks updated.
04/01/2023	3.2	Director of Nursing MHPHDS	Minor addition under 6.7, use of non-tear garments and process.
20/04/2023	3.3	Director of Nursing MHPHDS	Minor addition page 4 section 6 : the shift coordinator will identify the periods of time allocated to nurse undertaking clinical observations level 1 for every shift to ensure their safety and wellbeing.
29/05/2024	4.0	Director of Nursing MHPHDS	Update of flowcharts for nursing clinical observation level 1, update of flowchart for medical review, update references and clinical processes to reflect current processes in relation to interhospital transfers

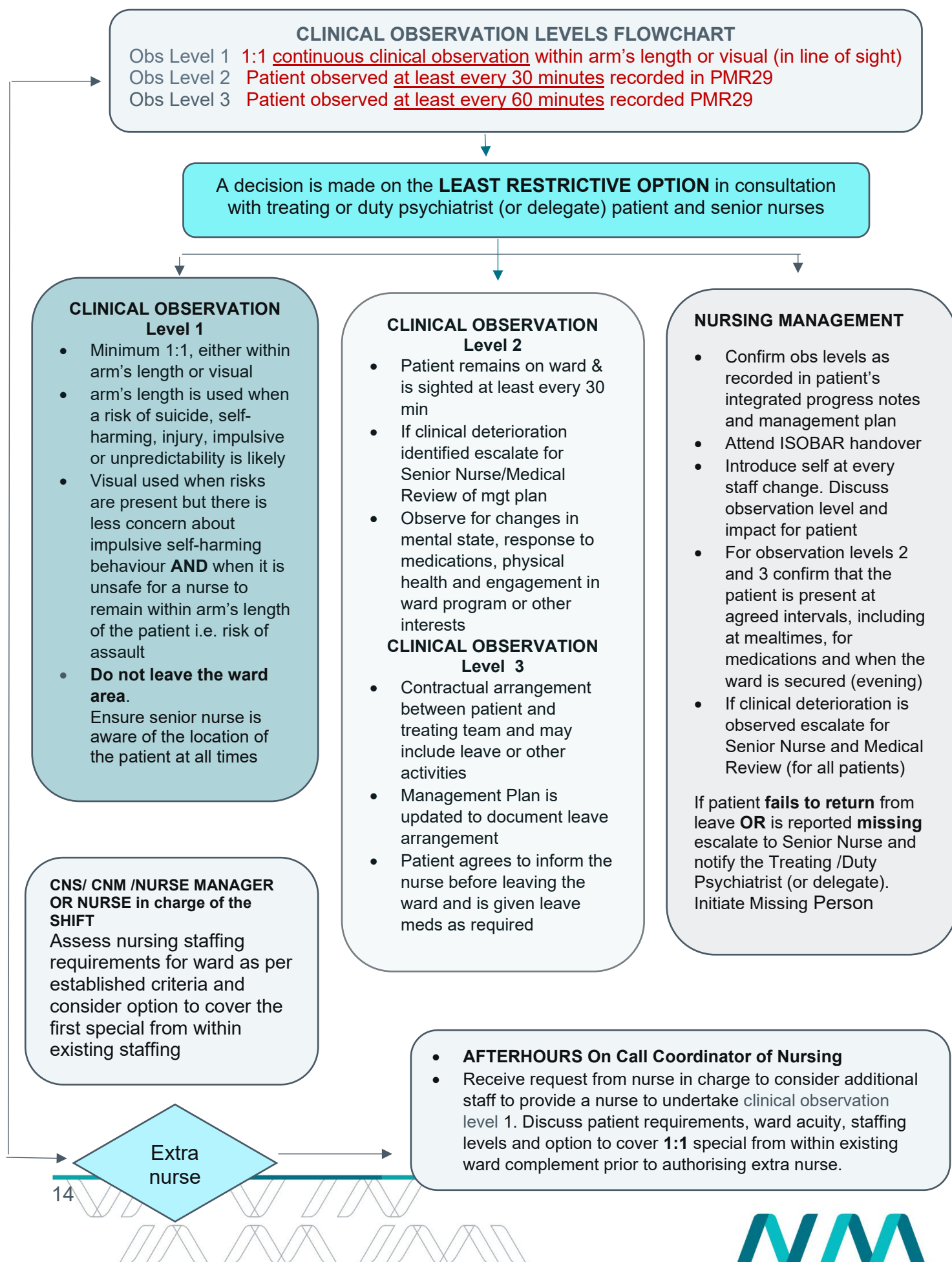
The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (**ISD Number: 3094**)

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Appendix 1: Clinical Observation Levels Flowchart



Appendix 2: Medical Review of Clinical Observation Level 1

MEDICAL REVIEW OF CLINICAL OBSERVATION LEVEL 1 FLOWCHART

Specify Clinical Observation level and whether within arm's length or visual

Conduct face-to-face review including risk assessment by **1100 hrs every 24 hrs**

FOLLOWING CONSULTATION LEAST RESTRICTIVE CLINICAL OBSERVATION LEVEL REQUIRED IS DECIDED

CLINICAL OBSERVATION LEVEL 1 1 to 7 DAYS

- Multidisciplinary input for the TSDP
- Reviewed (face-to-face) by the treating or Duty Psychiatrist (or medical delegate) **every 24 hours, by 1100 hours** including weekends.
- The type of clinical observation level 1 and the reason for the decision clearly documented daily in integrated progress notes.
- Approval for clinical observation level 1 for a period of 1 to 7 days is by **Treating or Duty Psychiatrist** (or delegate)

CLINICAL OBSERVATION LEVEL 1 8 to 14 DAYS

- Multidisciplinary input for the TSDP
- Reviewed (face-to-face) by the treating or duty Psychiatrist (or medical delegate) every 24 hours by 1100 hours including weekends.
- The type of clinical observation level 1 and the reason for the decision is clearly documented daily in integrated progress notes.
- A review will occur when it is deemed necessary to continue CL1 observation level for 8 to 14 days. The review will include the Treating Psychiatrist (or delegate), CNS, the Head of Service (HOS) and relevant MDT members.
- Approval for clinical observation level 1 is by **Treating or Duty Psychiatrist** (or delegate)

CLINICAL OBSERVATION LEVEL 1 15 to 28 DAYS

- Multidisciplinary input for the TSDP
- Reviewed (face-to-face) by the treating or duty Psychiatrist (or medical delegate) every 24 hours by 1100 hours including weekends.
- The type of clinical observation level 1 and the reason for the decision clearly documented daily in the integrated progress notes.
- A review will occur weekly when it is deemed necessary to continue the clinical observation level 1 for 15 days to 28 days.
- The review will include Treating Psychiatrist, Head of Service (HOS), CoN, Co-Director (Medical) and members of the MDT

CLINICAL OBSERVATION LEVEL 1 continuing beyond 28 days (>28 days)

- Multidisciplinary input for the TSDP by clinicians involved in their care.
- Review face-to-face by Treating/ Duty Psychiatrist (or delegate) with clinical obs level 1 and type and reason for this decision documented in the progress notes and Management Plan.
- Approval for clinical observation level 1 greater than 28 days is by **HOS** in consultation with Medical Co-Director to ensure that all available options are considered.
- Continuation **>28** days may require review by the Medical Co-Director and Dir of Nursing



Appendix 3: Specials and Clinical Observation Report PMR29 (Pg1)

NORTH METROPOLITAN AREA HEALTH SERVICE MENTAL HEALTH		Surname		Sex	U.R. No
Hospital		Forenames		D.O.B.	
SPECIALS AND CLINICAL OBSERVATION REPORT Appendix 1 (updated October 2011)		Address			
		Ward	Registrar	Consultant	
		Use Patient I.D. Label when available			
Instructions: • Note the time the observation is conducted • Record the patient's whereabouts • Record the patient's respiration rate • Assess the patient's mental state to identify cues of increased risk and document action to be taken as a result of observations • Report immediately any clinical deterioration to the Shift Coordinator/Nurse in Charge • Ensure patient is safe until reviewed by treating or duty psychiatrist (or medical delegate) • Document in the patient's Progress Notes • If patient not sighted, report immediately to the Shift Coordinator/Nurse in Charge • Document your observations and interactions in the patient's Progress Notes • New form must be completed every shift					
DATE:	WARD:	LEGAL STATUS:	Level of Clinical Risk (CRAM):		
Level of Observation and Frequency of Documentation:		Reason for Observation:			
Description of patient and clothing: (Update description of clothing if patient changes apparel during the shift)					
INITIAL OBSERVATIONS ORDER (only) Observation initiated by: Print Name: Designation: Time: Authorised by Treating/Duty Psychiatrist or Medical Delegate: Print Name: Designation: Time:			EACH SHIFT REVIEW by CNS or CNM or Delegated Senior Nurse: Print Name: Designation: Time: Level 1 Obs Face to Face Daily (by 1100 hrs) REVIEW by Treating/Duty Psychiatrist or Medical Delegate: Print Name: Designation: Time:		

PMR29 SPECIALS AND CLINICAL OBSERVATION REPORT

Appendix 3: Specials and Clinical Observation Report PMR29 (Pg2)

[illegible]

Attach ADR Sticker		
ALLERGIES & ADVERSE DRUG REACTIONS (ADR) ☐ Nil known ☐ Unknown (tick appropriate box or complete below)		
Drug (or other)	Reaction / Type / Date	Initials
Sign	Print	Date

Sex ☐ M ☐ F

STEP 1	Medicine (Print generic name)		Dose	Route	Max dose/24 hrs	Check arousal rating at: Oral 60 & 90 minutes IM 30 & 60 minutes
Date: _/_/_						
	Prescriber signature	Print Name	Contact	Pharmacy		Imprest S4R
STEP 2	Medicine (Print generic name)		Dose	Route	Max dose/24 hrs	Check arousal rating at: Oral 60 & 90 minutes IM 30 & 60 minutes
Date: _/_/_						
	Prescriber signature	Print Name	Contact	Pharmacy		Imprest S4R
STEP 3	Medicine (Print generic name)		Dose	Route	Max dose/24 hrs	Check arousal rating at: Oral 60 & 90 minutes IM 30 & 60 minutes
Date: _/_/_						
	Prescriber signature	Print Name	Contact	Pharmacy		Imprest S4R
STEP 4	Medicine (Print generic name)		Dose	Route	Max dose/24 hrs	Check arousal rating at: Oral 60 & 90 minutes IM 30 & 60 minutes
Date: _/_/_						
	Prescriber signature	Print Name	Contact	Pharmacy		Imprest S4R

[illegible]

PMR60D AGITATION AND AROUSAL PRN MEDICATION CHART